

SWASTHYA KATHA — FIELD EDITION

DRAFT — PENDING CLINICAL SIGN-OFF

INDIA HIV/AIDS ALLIANCE • NACO NATIONAL AIDS & STD CONTROL PROGRAMME (NACP
PHASE V)

Swasthya Katha

STI & Syphilis Field Handbook

A simple, step-by-step ready reckoner for outreach workers, peer educators,
coordinators and clinic staff in prisons and closed settings

Look. Match. Act.

1

Look at what the person
shows or tells you

2

Match it to one box in this
book

3

Give the matching kit, or
send them onward — never
guess

Not sure which box fits? Don't guess.

Call your medical officer or the helpline on the last page. That is always the right choice.

This is a working draft. Every dose in this book must be checked by your
programme's clinical lead against the current official NACO guidelines before it is
printed and used in the field.

SECTION 1 — BEFORE YOU START

SWASTHYA KATHA FIELD HANDBOOK

The 7 Kits, At a Glance

Every kit has a color. Learn the colors first — they matter more than the names.

1 • Grey

Discharge / leaking fluid from the private parts

2 • Green

Unusual vaginal discharge, itching

3 • White

A sore or ulcer on the private parts

4 • Blue

Same as White, if allergic to penicillin

5 • Red

Painful blisters that break open

6 • Yellow

Lower belly pain in a woman

7 • Black

A painful swollen lump in the groin

Who gives the kit: this page tells you which color matches which sign. Who is allowed to hand over the tablets, and how much, follows your own programme's training and supervision rule — this book supports that training, it doesn't replace it.

Quick Word List

Discharge — fluid or liquid coming from the private parts that isn't normal.

Ulcer / sore — a break or wound in the skin that doesn't heal quickly.

Bubo — a swollen, sometimes painful lump in the groin.

Presumptive treatment — giving the medicine because the pattern matches, even before a lab result comes back.

Partner notification — telling a person's recent partner(s) so they can get checked too.

Refer — send the person to a doctor or the nearest clinic instead of treating them yourself.

Intake Screening

A correction worth knowing: the list below is a **TB (tuberculosis) symptom check**, not an STI check — it just happens to be done at the same time, at intake. Keep the two separate in your head: TB symptoms below, STI/RTI signs on the pages that follow.

TB symptom check — ask about all ten

1. Cough

6. Out of breath easily

2. Fever

7. Not wanting to eat

3. Sweating heavily at night

8. Feeling very tired

4. Losing weight without trying

9. Swelling in the neck

5. Chest pain

10. Coughing up blood — refer today, don't wait

Alongside this, offer the **HIV + Syphilis quick test** — one finger-prick, one result in about 20–30 minutes. A positive result on either goes to the clinic the same day.

SECTION 2 — SPOT IT (1/4)

SORES ON THE PRIVATE PARTS

A Sore or Ulcer — What Kind?

The single most useful question: **does it hurt?**

One sore, no pain

Likely: Primary Syphilis

Give Kit 3 · White

SPOT IT

- Just one sore, not several
- Clean, smooth edges
- Firm to the touch
- Doesn't hurt at all



♂ Primary Chancre (Male): Dry, painless ulcer on glans



♀ Primary Chancre (Female): Painless ulcer on vulva

Kit 3

Give one injection + one tablet, as trained.

EXACT DOSE — FOR THE DISPENSING OFFICER TO CONFIRM

Inj. Benzathine Penicillin G 2.4 MU IM, single dose + Tab. Azithromycin 1 g, single dose. Penicillin-allergic → Kit 4 (Doxycycline 100mg BD ×15 days + Azithromycin 1g). Not for pregnant clients.

Also do: ask about partners from the last 3 months — they need checking too.

Remember: because it doesn't hurt, people often don't mention it. Always ask directly.

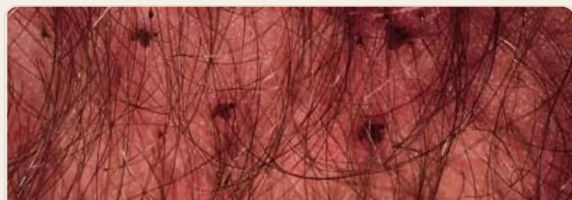
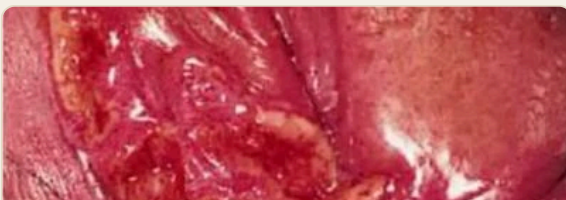
One or more sores, very painful

Likely: Chancroid

Give Kit 3 · White

SPOT IT

- One or more sores
- Ragged, messy edges
- Soft, not firm
- Hurts a lot
- Sometimes a painful swollen lump in the groin



Chancroid: Multiple deep, ragged, painful ulcers

Pediculosis Pubis: Crab lice and nits on pubic hair

Kit 3**Same kit as the painless sore above — it covers both at once.**

Safety rule: if there's a swollen lump, never cut or squeeze it. If it's soft and full of fluid, only a trained person should draw fluid out, through healthy skin only.

Rash on palms and soles, or patchy hair loss

Likely: Secondary Syphilis

Give Kit 3 · White

SPOT IT

- Rash spreads across the body, including palms and soles
- Patches of hair missing
- Sores or bumps in the mouth
- Flat, wart-like bumps in warm skin folds



Secondary Syphilis: Copper-red rash on palms



Condyloma Lata: Highly infectious moist lesions

Kit 3**Ask the dispensing officer — the exact schedule depends on how long the person has been infected.**

EXACT DOSE — CONFIRM STAGING WITH THE CLINICIAN FIRST

Under 1 year infected: single injection, same as Kit 3 above. Over 1 year, or unknown: same injection, once a week for 3 weeks. Staging must be confirmed by the treating clinician — don't guess it from this page.

Also do: ask about partners from the last 12 months.

SECTION 2 — SPOT IT (2/4)

DISCHARGE OR LEAKING FLUID

Discharge — What Kind?

Thick, heavy, yellow-white discharge

Likely: Gonorrhoea

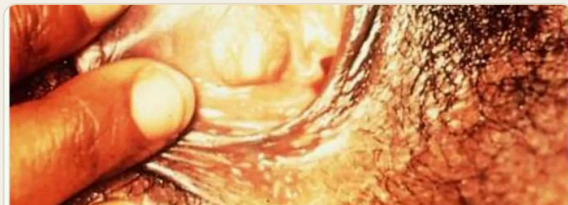
Give Kit 1 • Grey

SPOT IT

- Thick and heavy — hard to miss in men
- Burning or pain when passing urine
- In women, may show almost nothing — don't rule it out just because there are no signs



♂ Male Urethritis: Copious creamy purulent discharge



♀ Female Urethritis: Purulent discharge at meatus

Kit 1 One tablet of each kind, taken together, once.

EXACT DOSE

Tab. Cefixime 400 mg single dose + Tab. Azithromycin 1 g single dose.

Also do: treat the partner too, even if they show nothing — ask about partners from the last 60 days.

Remember: a lot of women carry this with no signs at all. If her partner has it, treat her too — don't wait for her to show something.

Thin, watery discharge — or none at all

Likely: Chlamydia

Give Kit 1

SPOT IT

- Thinner and lighter than the gonorrhoea discharge
- Very often — no discharge noticed at all



♂ Chlamydial Urethritis: Thin, clear-to-white discharge



LGV Bubo: Unilateral tender groin swelling

Kit 1 Same kit as gonorrhoea above — one dose covers both.

Painful swollen lump in the groin, with or without a small sore first

Give Kit 7 • Black

*Likely: LGV***SPOT IT**

- A firm, painful, growing lump in the groin
- Sometimes a tiny sore came first, and healed unnoticed

EXACT DOSE

Tab. Doxycycline 100mg BD ×21 days (42 tabs) + Tab. Azithromycin 1g single dose.

Safety rule: never cut or drain this lump. If needed, only a trained person draws fluid through healthy skin nearby.

SECTION 2 — SPOT IT (3/4)

BLISTERS AND ITCHING

Blisters & Itching

Small painful blisters, then open sores

Likely: Genital Herpes

Give Kit 5 · Red

SPOT IT

- Clusters of small blisters
- They break open into shallow, painful sores
- First time is usually worse — may come with fever
- Comes back sometimes — that's normal for this condition, not a new infection each time



♂ Genital Herpes (Male): Clustered fluid-filled vesicles



♀ Genital Herpes (Female): Painful coalescing ulcers

Kit 5

Tablets three times a day, for one week.

EXACT DOSE

Tab. Acyclovir 400mg, 3 times a day for 7 days (21 tablets) – for a first episode. A repeat episode may need a shorter course – check with the clinician.

Remember: this doesn't fully cure — it eases the episode. Say that plainly so the person isn't confused later.

Soft, painless, cauliflower-shaped bump

Genital warts (HPV)

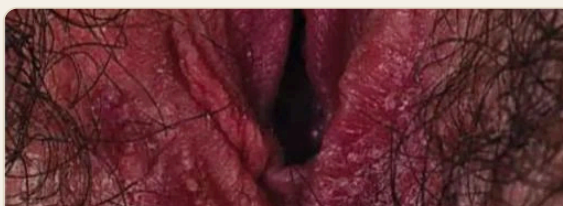
Clinical Care

SPOT IT

- Painless
- Bumpy, rough surface — like a small cauliflower
- Can be one or a cluster



♂ Genital Warts (Male): Giant verrucous lesions



♀ Genital Warts (Female): Fourchette verrucous lesions

No tablet kit for this one — refer to Suraksha Clinic for Podophyllin / Cryotherapy. Offer women cervical screening.

SECTION 2 — SPOT IT (4/4)

SIGNS IN WOMEN

Vaginal Discharge & Belly Pain

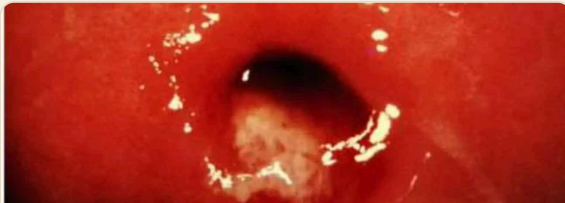
Unusual vaginal discharge, itching or smell

Vaginitis

Give Kit 2 · Green

SPOT IT

- Discharge that looks, smells, or feels different from usual
- Itching or soreness
- No belly pain, no fever



Endocervicitis: Purulent exudate & friable cervix



Bartholin's Abscess: Fluctuant tender labial mass

Kit 2 Two tablets, taken together, once.

EXACT DOSE

Tab. Secnidazole 2g single dose + Tab. Fluconazole 150mg single dose.

Lower belly pain, may have fever

Pelvic Inflammatory Disease (PID)

Give Kit 6 · Yellow

SPOT IT

- Pain low in the belly, not just discharge
- Sometimes fever
- Needs treatment quickly — don't wait and see

Kit 6 One tablet once, plus two more medicines twice a day for two weeks.

EXACT DOSE

Tab. Cefixime 400mg single dose + Tab. Metronidazole 400mg BD ×14 days (28 tabs) + Cap. Doxycycline 100mg BD ×14 days (28 tabs).

Also do: give the male partner Kit 1, even without symptoms.

Refer if: high fever, can't keep tablets down, or pain is severe — this needs a clinician, not just the kit.

SECTION 3 — QUICK LOOKUP TABLE

ALL 7 KITS, ONE PAGE

Kit Master Matrix

Note: NACO's system runs Kit 1 through Kit 7. Two extra conditions (warts, lice) are treated but don't have their own numbered kit.

Kit	Color	Used for	Contains	Partner rule
1	Grey	Discharge (gonorrhoea + chlamydia)	Azithromycin 1g (1) + Cefixime 400mg (1)	Treat, 60 days
2	Green	Vaginal discharge	Secnidazole 2g (1) + Fluconazole 150mg (1)	If symptomatic
3	White	Genital ulcer (syphilis + chancroid)	Benzathine Penicillin 2.4 MU (1) + Azithromycin 1g (1)	3 / 12 months
4	Blue	Genital ulcer — penicillin allergy	Doxycycline 100mg BD ×15d (30) + Azithromycin 1g (1)	Same as Kit 3
5	Red	Genital herpes, first episode	Acyclovir 400mg TDS ×7d (21)	Counselling
6	Yellow	Pelvic Inflammatory Disease	Cefixime 400mg (1) + Metronidazole 400mg BD ×14d (28) + Doxycycline 100mg BD ×14d (28)	Partner → Kit 1
7	Black	Lymphogranuloma Venereum	Doxycycline 100mg BD ×21d (42) + Azithromycin 1g (1)	Extended tracing

Before this leaves draft: every dose and duration above must be checked by your clinical lead against the current NACO National Technical Guidelines, and signed below.

Clinical reviewer:

[name, role]

Date reviewed:

[date]

Guideline checked:

[e.g. NACO 2024]

Next review due:

[date]

SECTION 4 — TALKING TO PEOPLE

RIGHTS, MYTHS & SIMPLE ANSWERS

Counselling & Common Myths

What every person has a right to

Under the HIV and AIDS Act, 2017: their result is private, they must agree before being tested, and no one can be treated worse because of their status. Explain this before testing, in plain words, every time.

Telling partners, without breaking trust

With the person's permission, help them think through who else might be at risk — but never say their name or result to that person without separate permission.

Four things people often believe that aren't true

"You can tell just by looking."

Truth: many people show nothing at all. Only a test tells you for sure.

"You can catch it from a cup or a toilet."

Truth: it spreads through sex or blood, not sharing everyday things.

"Getting tested means everyone finds out."

Truth: it's private. Nobody has to know unless the person chooses to tell them.

"The signs went away, so it's gone."

Truth: signs can settle before the infection does. Finish every tablet in the kit.

Where to send someone

Nearest Suraksha Clinic: [name, address, phone]

Nearest ICTC: [name, address, phone]

Nearest ART Centre: [name, address, phone]

Medical officer on call: [name, phone]

1097

NACO National AIDS Helpline — free, from any phone, anywhere in India.

Fill in the boxes above with your facility's real contacts before this is printed and handed out. A wrong number helps no one.

When in Doubt

Refer, don't guess.

Every kit on the wall below is a starting point, not a final answer. If two boxes seem to match, or none do, send the person to the clinic today.

1 • Grey

Discharge

2 • Green

Vaginal discharge

3 • White

Painless or painful sore

4 • Blue

Sore, penicillin-allergic

5 • Red

Painful blisters

6 • Yellow

Belly pain (women)

7 • Black

Groin lump

This handbook was built for people doing real work in hard settings, with limited time and limited privacy. It is a starting point, written in plain language on purpose — it is not a replacement for training, supervision, or the judgement of a qualified clinician. Use it, question it, and help improve it.

Document control

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Status: Not yet cleared for field distribution